

SUPERIOR COURT OF CALIFORNIA
COUNTY OF VENTURA

Tentative Ruling

**2026CUPP066531: DEBBIE READING, et al. vs MD HYDRATION FRANCHISE, INC.,
A CALIFORNIA CORPORATION, DBA THE HYDRATION ROOM, et al.**

08/17/2026 in Department 41

Demurrer for Brett Michael Florie DO, McKensie Archer, Emily Siderman

Defendants Brett Michael Florie, D.O.; McKensie Archer and Emily
Siderman's Demurrer to Plaintiff's Complaint (*opposed*).

Defendants Brett Michael Florie, D.O.; McKensie Archer and Emily Siderman's
Motion to Strike Portions of Plaintiff's Complaint (*opposed*).

Defendants Brett Michael Florie, D.O.; McKensie Archer and Emily *Siderman's Demurrer to Plaintiff's Complaint is SUSTAINED in part and OVERRULED in part.*

The demurrer to the 1st, 2nd, 3rd, 4th, 7th, and 9th causes of action are overruled. The demurrer to the 5th, 6th and 8th causes of action are sustained with leave to amend

Defendants Brett Michael Florie, D.O.; McKensie Archer and Emily Siderman's *Motion to Strike Portions of Plaintiff's Complaint is GRANTED in part and MOOT in part.*

The motion to strike *is granted with respect to any claim for punitive damage relating to the professional negligence claims.* The motion to strike is moot in all other respects considering the ruling on the demurrer.

Plaintiff may file and serve a First Amended Complaint within 20 days of this ruling.

Notice to be provided by the moving Defendants.

1st Cause of Action for Professional Negligence/Medical Malpractice: Plaintiff1 alleges that no physician nor any nurse practitioner conducted a prior assessment of Plaintiff on May 22, 2025. However, there is no allegation that Defendants Florie or Archer provided any care or treatment to Plaintiff. Their liability, if any, would be derivative from Nurse Siderman, but if insufficient facts are stated to establish Siderman's liability in the first instance, they would have no vicarious liability. In addition, the claim that Archer failed to develop standardized procedures for prior patient assessment is not a recognized duty by the Board of Registered Nurses. Defendant Florie is also not required to develop a prior nursing assessment before the injection.

The pleading is also uncertain as to who assessed Plaintiff on May 22, 2025, and as to the scope of what was done to plaintiff before she received her injection. The Complaint's arguments regarding res ipsa loquitor or gross negligence are also unavailing.

2nd Cause of Action for General Negligence/Negligent Business Operations and 3rd Cause of Action for Negligence Per Se: The second cause of action is, in essence, Plaintiff's attack on the

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business model followed by wellness clinics. The third cause of action is based upon Plaintiff's belief that certain Federal and California state laws prohibit their operations. However, nothing shows that the administration of glutathione is improper or that it is a dangerous drug.

4th Cause of Action for Lack of Informed Consent: This cause of action requires that Plaintiff be warned of the specific nerve that could be injured by the injection. However, this is not the standard. The consent form which states that there may be nerve injury and that she may experience shooting pains down the leg is sufficient notice.

5th Cause of Action for Battery: Battery is reserved for circumstances when a doctor performs an operation to which the patient has not consented. The 5th cause of action fails this legal test. Plaintiff admits that she expected to receive an injection of glutathione into her right butt cheek, but that it would be injected into the anatomically correct zone of her buttock. That Plaintiff pleads the injection fell outside of the anatomically safe zone does not constitute "an operation to which the patient has not consented" nor does it constitute some new and additional treatment, nor an intentional deviation from the plan.

6th Cause of Action for Fraud/Intentional Misrepresentation/Alternatively, Negligent Misrepresentation: The Complaint fails to meet the heightened pleading requirement for pleading fraud.

7th Cause of Action for Negligent Supervision, Training, and Retention: Plaintiff's seventh cause of action for negligent supervision/retention is also legally deficient. It fails to allege facts to show that Defendants Florie or Archer knew Sideman was "incompetent or unfit."

8th Cause of Action for Violation of Business and Professions Code §17200: This cause of action has no supporting facts. All that is pleaded is based on information and belief.

9th Cause of Action for Loss of Consortium: This cause of action is deficient since it relies on fraud, misrepresentation and other tort theories that are not personal injury based.

The Motion to Strike seeks to strike all causes of action for all the reasons in the Demurrer.

Moreover, and specifically, moving party seeks to strike the punitive damages allegations on the grounds that they are being sought being sought against health care providers without obtaining permission from the court as required by Code of Civil Procedure section 425.13.

The Declaration of Mark L. Kiefer stated that he sent some correspondence to Plaintiff's counsel on June 12, 2026, detailing several concerns about the Complaint. Plaintiff's counsel sent a response on June 13, 2026, and he sent a response on June 19, 2026. This is sufficient to meet the requirements of Code of Civil Procedure, section 430.41, subdivision (a).

The elements of a Plaintiff's medical negligence cause of action must allege "(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting

from the professional's negligence.” (*Belfiore-Braman v. Rotenberg* (2018) 25 Cal.App.5th 234, 238, fn. 3.)

The demurrer argues that the Complaint fails to plead the element of duty. As noted, the duty required to be pleaded is “the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise.” (*Hanson v. Grode* (1999) 76 Cal.App.4th 601, 606.)

The Complaint alleged that Defendant Siderman, a registered nurse, was “obligated to exercise the level of skill, knowledge, and care that a reasonably careful registered nurse would use in similar circumstances, including proper selection of anatomically safe injection sites, recognition of and appropriate response to adverse patient reactions occurring during or immediately following an injection procedure, accurate and complete clinical documentation of patient complaints, and escalation to a supervising provider when a patient reports symptoms consistent with an abnormal reaction or nerve involvement.” (Compl. ¶ 33.)

The Complaint further alleged that Defendant Archer is a nurse practitioner and “s obligated to exercise the level of skill, knowledge, and care that a reasonably careful family nurse practitioner would use in similar circumstances, including developing and enforcing written standardized procedures governing the administration of injectable medications by nursing staff under her supervision, ensuring that pretreatment patient assessments were performed, and providing adequate real-time and retrospective oversight of clinical practice at the Thousand Oaks Clinic.” (Compl. ¶ 34.)

As to Defendant Florie, a DO, the Complaint alleged that he “was at all relevant times obligated to exercise the level of skill, knowledge, and care that a reasonably careful osteopathic physician serving as a medical director and supervising physician would use in similar circumstances, including establishing, implementing, and enforcing protocols governing the safe administration of injectable medications at facilities under his clinical direction, and ensuring that the practice operated in compliance with applicable California licensing and prescribing statutes.”

Thus, the Complaint sufficiently pleads duty by alleging that Defendants were required to use skill, prudence and diligence as other members of the professional community.

The demurrer cites to the “Explanation of the Scope of RN Practice Including Standardized Procedures” published by the Board of Registered Nurses for the State of California (Rev. 01/2011), as the basis for finding that the duty element was not sufficiently pleaded. (Demurrer, p. 8:21-22). However, this document is not pleaded and no request for judicial notice was made. Since no extrinsic evidence may be considered on demurrer (*Ion Equip. Corp. v. Nelson* (1980) 110 Cal.App.3d 868, 881), any argument based on this document will not be considered. Therefore, the demurrer for failure to state a claim is overruled.

Defendant also argues that the pleading is uncertain as to who assessed Plaintiff and the scope of what was done before the injection.

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A demurrer for uncertainty will be sustained only where the complaint is so bad that defendant cannot reasonably respond. (See *Khoury v. Maly's of Calif., Inc.* (1993) 14 Cal.App.4th 612, 616.) These are matters that can be clarified during discovery. Therefore, the demurrer for uncertainty is overruled.

Moreover, arguments regarding whether the principles of *res ipsa loquitur* and/or gross negligence apply relate to a portion of the allegations regarding this cause of action. (Compl. ¶¶40-41). It is not determinative of the entire cause of action. “Ordinarily, a general demurrer does not lie as to a portion of a cause of action, and if any part of a cause of action is properly pleaded, the demurrer will be overruled.” (*Fire Ins. Exchange v. Superior Court* (2004) 116 Cal.App.4th 446, 452.) Therefore, the arguments regarding the sufficiency of the 1st cause of action’s reliance on these principles is only a challenge to a portion of the cause of action and cannot be sustained.

Based on the foregoing, the demurrer is overruled.

“A complaint for damages for negligent injury to person or property must allege: (1) defendant's legal duty of care toward plaintiff; (2) defendant's breach of duty, i.e., the negligent act or omission; (3) injury to plaintiff as a result of the breach, i.e., proximate or legal cause; and (4) damage to plaintiff . . . The negligence element may be generally pleaded; the breach of duty of care may be alleged by stating the act was negligently done.” (*Pultz v. Holgerson* (1986) 184 Cal.App.3d 1110, 1116-1117.)

The negligence per se doctrine, as codified in Evidence Code section 669, creates a presumption of negligence if four elements are established: “(1) the defendant violated a statute, ordinance, or regulation of a public entity; (2) the violation proximately caused death or injury to person or property; (3) the death or injury resulted from an occurrence of the nature of which the statute, ordinance, or regulation was designed to prevent; and (4) the person suffering the death or the injury to his person or property was one of the class of persons for whose protection the statute, ordinance, or regulation was adopted.” (*Galvez v. Frields* (2001) 88 Cal.App.4th 1410, 1420.)

The demurrer argues that these causes of action are an attack on Plaintiff’s business model and the belief that certain Federal and California laws prohibit the operations. These arguments are not arguments regarding the sufficiency of the allegations in the Complaint but attempts to argue the merits of the causes of action using extrinsic evidence such as information on the FDA website and the factual issue of whether glutathione, by itself, is a drug. As noted above, a “speaking demurrer” is improper. Therefore, the demurrer to these causes of action is overruled.

“In the medical context, the law has developed the doctrine of informed consent, which requires a patient's consent be an informed consent to be effective and imposes a duty on the physician to provide material information about any proposed treatment, such as risks and alternative procedures. [Citations]. Under the informed consent doctrine, ““the patient must have the capacity to reason and make judgments, the decision must be made voluntarily and without coercion, and the patient must have a clear understanding of the risks and benefits of the proposed treatment alternatives or nontreatment, along with a full understanding of the nature of

the disease and the prognosis.’ “‘ [Citation].” (*Conte v. Girard Orthopaedic Surgeons Medical Group, Inc.* (2003) 107 Cal.App.4th 1260, 1267.)

“Because a patient relies upon her physician's greater medical knowledge when seeking medical treatment, the physician has a fiduciary-like duty to obtain his patient's informed consent regarding which course of treatment to pursue. [Citations]. A physician who fails to do so is liable for negligence. [Citations]. In imposing this duty, the doctrine of informed consent ‘injects into the established framework of negligence a concern with patient choice that would otherwise be absent.’ [Citation].” (*Flores v. Liu* (2021) 60 Cal.App.5th 278, 292.)

“To comply with the duty to obtain a patient's informed consent, a physician must ‘disclose to the patient all material information—that is, ‘information which the physician knows or should know would be regarded as significant by a reasonable person in the patient's position when deciding to accept or reject a recommended medical procedure.’ [Citations]. This standard focuses on what an objective, reasonably prudent person’ in the patient's shoes would want to know and is therefore not dictated by whatever ‘custom’ physicians in the relevant medical community follow when making disclosures.” (*Flores v. Liu, supra*, 60 Cal.App.5th at pp. 292-293.)

The Complaint alleged: “Defendants failed to obtain Ms. Reading’s specific informed consent for the procedure as actually performed on May 22, 2025. While the generic consent forms enumerated general risks including ‘shooting pains down your legs’ and ‘nerve injury,’ they did not disclose the specific material risks actually relevant to Ms. Reading’s procedure: (a) the substantially elevated risk of sciatic nerve injury associated with high-dose gluteal intramuscular injections in patients with minimal gluteal muscle mass and lean body habitus; (b) the specific compounded, non-FDA approved nature of the glutathione preparation, including its volume, concentration, and potential additional components; (c) the absence of any prior medical assessment or physician authorization before the injection; and (d) the risk of Complex Regional Pain Syndrome Type 2, a permanent, chronic neuroimmune condition recognized as among the most painful syndromes known to medicine, characterized by allodynia, autonomic dysregulation, and potentially lifelong functional impairment. No consent form obtained by THR disclosed or described the risk of CRPS, its mechanism, or its prognosis. A reasonable person who understood that this injection could trigger a permanent, debilitating chronic pain condition of this nature, as opposed to a generic ‘nerve injury’ that typically resolves, would have required substantially more information before consenting and may well have declined the procedure.” (Compl. ¶ 59.)

The demurrer argues that the Complaint fails to sufficiently plead this cause of action because no case in California has required a consent form for an injection to say more than “nerve injury.” However, the standard, as noted above, is what an objective, reasonable ‘prudent person’ in the patient's shoes would want to know, and is therefore not dictated by whatever ‘custom’ physicians in the relevant medical community follow when making disclosures.” (*Flores v. Liu, supra*, 60 Cal.App.5th at pp. 292-293.) Whether a general identification of “nerve injury” is sufficient or not is dependent on facts outside the Complaint such as what the physician knows or should know as significant in obtaining informed consent. Therefore, determination of this matter cannot be made on demurrer.

Defendant also argues that causation was not pleaded since the Complaint fails to plead and prove that had Defendants specified “sciatic nerve” that Plaintiff would have refused the shot. As this is on demurrer, “proof” is not necessary. With respect to pleading, the Complaint pleads: “[a] reasonable person in Ms. Reading’s position, fully informed of the specific risks identified above, would not have consented to this procedure as performed.” (Compl. ¶62.) Accordingly, the demurrer is overruled.

An action “should be pleaded in negligence” when the doctor performs an operation to which plaintiff consents, but without disclosing sufficient information about the risks inherent in the surgery. (*Cobbs v. Grant* (1972) 8 Cal.3d 229, 240-241.) “The battery theory should be reserved for those circumstances when a doctor performs an operation to which the patient has not consented.” (*Id.* at p. 240.)

“‘When an action is based upon the theory of surgery beyond consent, the gist of such action is the unwarranted exceeding of the consent. This is a theory of technical battery.’ [Citation].” (*Conte v. Girard Orthopaedic Surgeons Medical Group, Inc.*, *supra*, 107 Cal.App.4th 1260, 1267.)

The Complaint alleged: “Defendant Sideman, acting within the course and scope of her employment by Defendant THR, administered to Ms. Reading a glutathione intramuscular injection into Ms. Reading’s right gluteal region. Ms. Reading consented to receive a glutathione injection administered in a clinically safe manner at a safe anatomical location. She did not consent to an injection placed outside the anatomically safe zone for gluteal intramuscular injections, the upper outer quadrant of the gluteus maximus, or to an injection administered without accounting for her physical characteristics and without any real-time supervision. The injection as actually administered by Defendant Sideman deviated materially from the procedure to which Ms. Reading consented, constituting an intentional touching of Ms. Reading’s person that exceeded the scope of her consent. Defendant THR is vicariously liable for this touching.” (Compl., ¶ 66.)

The allegation in the Complaint alleges that Plaintiff consented to receiving the glutathione injection, but only as to the safe administration of it in the proper area of the body. Thus, the allegations do not appear to be based on an action beyond the consent she provided, only that there was a negligent action by defendants in the administration of the injection. Therefore, the demurrer is sustained.

“Deceit, within the meaning of the last section, is: [¶] 1. The suggestion, as a fact, of that which is not true, by one who does not believe it to be true; [¶] 2. The assertion, as a fact, of that which is not true, by one who has no reasonable ground for believing it to be true; [¶] 3. The suppression of a fact, by one who is bound to disclose it, or who gives information of other facts which are likely to mislead for want of communication of that fact; or [¶] 4. A promise, made without any intention of performing it.” (Civ. Code, § 1710.)

“‘The well-known elements of a cause of action for fraud are: (1) a misrepresentation, which includes a concealment or nondisclosure; (2) knowledge of the falsity of the misrepresentation,

i.e., scienter; (3) intent to induce reliance on the misrepresentation; (4) justifiable reliance; and (5) resulting damages. [Citation]. The same elements comprise a cause of action for negligent misrepresentation, except there is no requirement of intent to induce reliance. [Citation]. In both causes of action, the plaintiff must plead that he or she relied on the misrepresentation.” (*Cadlo v. Owens-Illinois, Inc.* (2004) 125 Cal.App.4th 513, 519.)

“Each element in a cause of action for fraud or negligent misrepresentation must be factually and specifically alleged. [Citation]. The policy of liberal construction of pleadings is not generally invoked to sustain a misrepresentation pleading defective in any material respect. [Citation].” (*Cadlo v. Owens-Illinois, Inc.*, *supra*, 125 Cal.App.4th 513, 519.) “‘This particularity requirement necessitates pleading *facts* which show how, when, where, to whom, and by what means the representations were tendered.’ [Citation].” (*Lazar v. Superior Court* (1996) 12 Cal.4th 631, 645, emphasis in the original.)

The Complaint alleged that “[f]ollowing the administration of the injection and Ms. Reading’s immediate report of an abnormal sensation and pain, Defendant Siderman falsely represented to Ms. Reading that the abnormal sensation and pain were expected and that the injection could be painful due to the substance and dosage. Defendant Siderman, as a licensed registered nurse trained in the administration of intramuscular injections and the recognition of adverse injection events, knew at the time she made this representation that Ms. Reading’s reported symptoms, a ‘weird’ sensation and pain, were not a normal or expected response to a gluteal intramuscular injection and were instead the characteristic clinical presentation of sciatic nerve involvement. A trained RN informed in real time that a patient feels a ‘weird’ sensation following a gluteal injection would recognize this as a potential nerve injury event requiring immediate follow-up, clinical assessment, and escalation to a supervising provider. Defendant Siderman instead dismissed the complaint without asking a single follow-up question, not whether the sensation was bilateral or unilateral, not whether it was localized or radiating, not whether it was electric or merely sore, and made no inquiry into the character or severity of the complaint. Her failure to ask even the most basic follow-up questions in the face of a patient’s report of a “weird” sensation and pain, potential indicators of nerve involvement, demonstrates that she either knew her reassurance was clinically unsupported and made it anyway, or acted in reckless disregard for the truth.” (Compl. ¶ 70.) “Defendant Siderman subsequently made a further false and affirmative misrepresentation in THR’s medical records by documenting that the ‘patient tolerated injection well and no report of pain with injection.’ Defendant Siderman knew this entry was false at the time she made it, having been directly and contemporaneously informed by Ms. Reading, in the presence of her son, that the injection felt abnormal and was painful. (Compl. ¶ 71.) “These misrepresentations were made with the intent to induce Ms. Reading to believe she had not been injured, to prevent her from seeking immediate emergency medical care, and to create a false clinical record limiting THR’s exposure in any subsequent claim or litigation.” (Compl. ¶ 72.) “Ms. Reading reasonably relied on Defendant Siderman’s representation that her symptoms were expected. In reliance on that representation, she did not call 911 or request emergency medical evaluation at the clinic.” (Compl. ¶ 73.) “As a direct and proximate result of these intentional misrepresentations, Ms. Reading suffered harm including delayed diagnosis and treatment, aggravation of her injuries, and a permanently falsified medical record.” (Compl. ¶ 74.)

The allegations in the Complaint do not sufficiently plead scienter and intent to induce reliance on the misrepresentation. The allegations in paragraph 70 regarding “scienter” are speculative as to what a nurse in Defendant Sideman’s position would consider the meaning of a patient saying she felt something “weird.” In addition, there is insufficient allegation regarding intent to induce reliance when the assurance was that the sensation was expected and the alleged false documentation was not directed on Plaintiff. Therefore, the demurrer is sustained.

“California case law recognizes the theory that an employer can be liable to a third person for negligently hiring, supervising, or retaining an unfit employee. [Citation]. Liability is based upon the facts that the employer knew or should have known that hiring the employee created a particular risk or hazard and that harm materializes.” (*Doe v. Capital Cities* (1996) 50 Cal.App.4th 1038, 1054.)

The Complaint alleged: “Defendants THR and Florie had a duty to exercise reasonable care in the training, credentialing, supervision, and retention of nursing staff and nurse practitioners at the Thousand Oaks Clinic, and to implement and enforce policies adequate to protect patients from the known risks associated with intramuscular injection therapy, including proper anatomical site selection, documentation accuracy, adverse event recognition and reporting, and prior patient assessment.” (Compl. ¶79.) “Defendants THR and Florie breached this duty by, among other things: (a) failing to train nursing staff in proper injection site selection for gluteal intramuscular injections; (b) failing to implement or enforce documentation standards requiring accurate and complete recording of injection site specifics and patient complaints; (c) failing to establish, document, and enforce standardized procedures requiring a prior patient examination before the administration of injectable medications, or, if such procedures existed, failing to ensure compliance with them; (d) deploying Defendant Sideman, a nurse with limited post-licensure clinical experience, to independently perform high-dose compounded substance injections without adequate training, competency verification, or supervision protocols specific to gluteal injection site selection; (e) failing to ensure that Defendant Archer provided real-time, meaningful supervision of injection procedures and responses to adverse patient events; and (f) failing to conduct ongoing competency evaluations of nursing staff performing injections.” (Compl. ¶ 80.)

The allegations that Defendants failed to properly train the nursing staff and deploying Defendant Sideman, a nurse with limited post-injection training, appear to be sufficient to plead this cause of action. Therefore, the demurrer is overruled.

Private parties may pursue a claim under the Unfair Competition and False Advertising Laws (Bus. & Prof. Code, §§ 17200, et seq., 17500, set seq.) if they have “suffered injury in fact and has lost money or property as a result of a violation of this chapter.” (*Kwikset Corp. v. Superior Court* (2011) 51 Cal. 4th 310, 322.) “Injury in fact” means “an invasion of a legally protected interest which is (a) concrete and particularized ...; and (b) actual or imminent, not conjectural or hypothetical.” (*Ibid.*) “Proposition 64 requires that a plaintiff have ‘lost money or property’ to have standing to sue. The plain import of this is that a plaintiff now must demonstrate some form of economic injury.” (*Id.* at p. 323.)

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The Complaint alleged: “Business and Professions Code section 17200 prohibits any unlawful, unfair, or fraudulent business act or practice. Defendants engaged in unlawful, unfair, and fraudulent business acts and practices, including: (a) furnishing injectable medications to patients without prior examination or medical indication in violation of Business and Professions Code section 2242; (b) operating an injection clinic in which RNs administered intramuscular injections either without written standardized procedures or without adhering to written standardized procedures governing site selection, patient assessment, adverse response protocols, and documentation, in violation of Business and Professions Code sections 2725, subdivision (c) and 2836.1; and (c) falsifying patient medical records to conceal an adverse clinical event.” (Compl. ¶ 84.) “On information and belief, the absence of a prior assessment requirement was a systematic policy applied to all patients receiving injectable medications at the Thousand Oaks Clinic, constituting a pattern of unlawful business practice, not an isolated event. THR continues to operate the Thousand Oaks Clinic and offer injectable services to members of the public; absent injunctive relief, the unlawful practices described herein will continue to expose other patients to the same risks that caused Ms. Reading's injuries.” (Compl. ¶ 85.)

The allegations in the Complaint are conclusory and not supported by any facts. Therefore, the demurrer is sustained.

“‘[T]he right to recover for loss of consortium is founded on the relationship of marriage, and absent such a relationship the right does not exist.’ [Citation]. ‘A person who suffers a loss of consortium as the result of a negligent or intentional injury to his or her spouse is entitled to recover damages from the tortfeasor.’ [Citation].” (*Vanhooser v. Superior Court* (2012) 206 Cal.App.4th 921, 927.) “There are four elements to a cause of action for loss of consortium: ‘(1) a valid and lawful marriage between the plaintiff and the person injured at the time of the injury; [¶] (2) a tortious injury to the plaintiff's spouse; [¶] (3) loss of consortium suffered by the plaintiff; and [¶] (4) the loss was proximately caused by the defendant's act.’ [Citation].” (*Ibid.*)

The Complaint alleged that Plaintiff David Reading is the husband of Debbie Reading. (Compl. ¶ 3.) The Complaint further alleged “As a direct and proximate result of Defendants’ negligence, gross negligence, fraud, misrepresentation, and other wrongful conduct described herein, Plaintiff David Reading has been deprived of the companionship, affection, society, comfort, and services of his wife, Debbie Reading. Prior to the injury, the Readings shared an active and engaged family life. Since the injury, Ms. Reading has been largely unable to participate in the activities and responsibilities she previously shared with her husband, has required significant care and support, and the couple’s relationship has been severely and permanently altered by her pain, functional limitations, and psychological distress.” (Compl. ¶ 90.) Therefore, all required elements are pleaded. The fact that the cause of action may improperly allege fraud or other non-personal injury torts are matters subject to motion to strike and not demurrer.

Based on the foregoing, the demurrer to the 1st, 2nd, 3rd, 4th, 7th, and 9th causes of action are overruled. The demurrer to the 5th, 6th and 8th causes of action are sustained with leave to amend.

Any party, within the time allowed to respond to a pleading, may serve and file a motion to strike the whole or any part thereof. (Code Civ. Proc., § 435, subd. (b)(1); Cal. Rules Court, rule 3.1322, subd. (b).) The Court may, upon motion, or at any time in its discretion and upon terms it deems proper: (1) strike out any irrelevant, false, or improper matter inserted in any pleading; or (2) strike out all or any part of any pleading not drawn or filed in conformity with the laws of California, a court rule, or an order of the court. (Code Civ. Proc., § 436, subds. (a) & (b); *Stafford v. Shultz* (1954) 42 Cal.2d 767, 782.) When the defect which justifies striking a complaint is capable of being cured, the court should allow leave to amend. (*Perlman v. Municipal Court* (1979) 99 Cal.App.3d 568, 575.)

“A motion to strike out, or, as often termed, ‘a motion to strike,’ is traditionally used to reach certain kinds of defects in a pleading that are not subject to demurrer.” (5 Witkin, Cal. Proc. (5th ed. 2020) Plead § 1008.) “A motion to strike can also be used as a ‘scalpel’—to cut out any ‘irrelevant, false or improper’ matters inserted therein.” (Weil & Brown, Cal. Prac. Guide, Civil Proc. before Trial (The Rutter Group rev. June 2024) ¶ 7:177.) Nevertheless, “use of the motion to strike should be cautious and sparing” and should not be used as “a procedural ‘line-item veto’ for the civil defendant.” (*PH II v. Superior Court* (1995) 33 Cal.App.4th 1680, 1683.)

Pursuant to Code of Civil Procedure section 435.5, subdivision (a), before filing a motion to strike, the objecting party shall meet and confer in telephone or in person with the opposing party for the purpose of determining whether an agreement can be reached to resolve the objections to the pleading. The meet and confer shall be completed at least five (5) days before the responsive pleading is due. (Code Civ. Proc., § 435.5, subd. (a)(2).)

The motion to strike based on the sufficiency of each cause of action is discussed above.

The motion to strike the punitive damages allegation is properly granted as to the negligence claim (Compl., Prayer, ¶ 4) and moot as to the claim for punitive damages associated with the fraud claims (Compl. ¶¶ 75, 76) wherein the demurrer is sustained with leave to amend.

Code of Civil Procedure, section 425.13, subdivision (a) states: “In any action for damages arising out of the professional negligence of a health care provider, no claim for punitive damages shall be included in a complaint or other pleading unless the court enters an order allowing an amended pleading that includes a claim for punitive damages to be filed. The court may allow the filing of an amended pleading claiming punitive damages on a motion by the party seeking the amended pleading and on the basis of the supporting and opposing affidavits presented that the plaintiff has established that there is a substantial probability that the plaintiff will prevail on the claim pursuant to Section 3294 of the Civil Code. The court shall not grant a motion allowing the filing of an amended pleading that includes a claim for punitive damages if the motion for such an order is not filed within two years after the complaint or initial pleading is filed or not less than nine months before the date the matter is first set for trial, whichever is earlier.”

Although the Opposition to the Motion to strike argues that the punitive damage allegations against THR are not pleaded with respect to the ordinary malpractice or professional negligence claim, that is not clearly stated in the Complaint. Therefore, to the extent that this is claimed

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with respect to the professional negligence claim, compliance with Code of Civil Procedure, section 425.13 is required.

Based on the foregoing, the motion to strike may be granted as to the claim for punitive damages alleged with the professional negligence claim and moot with respect to the fraud claims.